

CANONICAL LESSON

Distance Is Not Access

Decision-centred foundations for accountable sector intelligence

Production Unit	PU-B07-C24
Notebook	V7_B_N04
Status	Controlled educational draft v0.1
Data boundary	Synthetic instructional data only

Central decision: Measure health-service access through travel time, affordability, availability, acceptability, quality, disability, and geographic equity.

Learning outcomes

Why this matters

Straight-line distance can create a false picture of access and hide systematic exclusion.

The analytical workflow is useful only when evidence, uncertainty, authority, and action are connected explicitly. The model supports professional judgement; it does not replace accountable institutional authority.

Worked cross-sector example

Two communities at equal distance have different access because roads, transport cost, opening hours, and service readiness differ.

Boundary: The example supports learning and review. It does not constitute an official statistic, eligibility determination, enforcement action, clinical judgement, or operational authorization.

Practice and reflection

- 1.
- Replace Euclidean with travel time.
- 2.
- Add facility capacity.
- 3.
- Define equity monitoring.

Answer key

- 1.
- Use a validated routable network with mode, season, closures, and border constraints.
- 2.
- Allocate catchments under service-specific capacity and waiting-time constraints.
- 3.
- Publish population-weighted access distributions by protected and geographic groups with uncertainty and disclosure controls.

Operational readiness checklist

- Purpose and decision owner are named.
- Evidence definitions, provenance, and lawful access are documented.
- Uncertainty and abstention are visible.
- Human review, escalation, monitoring, incident response, and retirement are assigned.
- Equity, accessibility, security, and sector-specific approval are complete.